



Society of Paediatric Anaesthesiologists of
Kenya

Strategy 2025-2030



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ACRONYMS & ABBREVIATIONS

CANECSA	College of Anaesthesiologists of East, Central, and Southern Africa
CEC	County Executive Committee
CME	Continuous Medical Education
CoG	Council of Governors
CPD	Continuous Professional Development
CUE	Commission for University Education
KDHS	Kenya Demographic and Health Survey
KMA	Kenya Medical Association
KMPDC	Kenya Medical Practitioners and Dentists Council
Kshs.	Kenya shillings
L4; L5; L6	Level-4; Level-5; Level-6
MMed	Master of Medicine degree
MoE	Ministry of Education
MoH	Ministry of Health
MoU	Memorandum of Understanding
No.	Number
p.a.	per annum
PA	Paediatric Anaesthesia
PAEDS	Paediatrics
SAFE	Safer Anaesthesia from Education
SO	Strategic Objective
SPAK	Society of Paediatric Anaesthesiologists of Kenya
SWOT	Strengths, Weaknesses, Opportunities, and Threats
WHO	World Health Organization

FORWARD

Chairperson's Message

EXECUTIVE SUMMARY

This 2025-2030 Strategy focuses on four key areas that will facilitate the operationalize of the Society's mission *"To Educate and Equip Anaesthesia Providers for Excellence in Paediatric Anaesthesia Care,"* and the attainment of its vision of *"Excellence in Paediatric Anaesthesia Provision Locally, Regionally, and Globally."* These four strategic pillars are: (i) Advocacy; (ii) Member Development; (iii) Enhanced Safety of Paediatric Patients; and (iv) Accessibility of Paediatric Anaesthesia Services. Each pillar has a desired outcome, which, respectively, are (i) better MoH & county governments' support for paediatric anaesthesia; (ii) increased number and capacity building of paediatric anaesthesiologists; (iii) compliance with WHO/KDHS 2022 standards for paediatric care; and (iv) paediatric anaesthesia services available at all Level-5 & Level-6 health facilities in Kenya.

Nine strategic objectives have been identified for implementation during the period of the Strategy in order to achieve the desired outcomes. These include seven enabler objectives, namely: *SO1 - Enhance Strategic Partnerships; SO2 - Improve Awareness of Paediatric Anaesthesia; SO3 - Enhance Welfare of Members; SO4 - Lobby National & County Governments for Increased Support for Paediatric Anaesthesia Services; SO5 - Enhance Training of Paediatric Anaesthesia Practitioners; SO6 - Broaden/Deepen Knowledge of Paediatric Anaesthesiologists; and SO7 - Grow the Number of Paediatric Anaesthesiologists.* The last objective will see the increase of the number of qualified paediatric anaesthesiologists from the current 20 to the targeted 60 by the end of the plan period.

Successful execution of these seven objectives will engender the realization of the two results objectives of *SO8 - Increase the Number of Facilities Adequately Equipped and Accredited to Offer Paediatric Anaesthesia; and SO9 - Enhance Oversight of Paediatric Anaesthesia Services,* which ultimately assure the safety of paediatric patients.

The implementation of the Strategy is expected to cost and estimated Kshs. 19.7 million, which will mainly be funded through the support of development partners and by member contributions. Regular monitoring and evaluation will be conducted to ensure the implementation of the Strategy remains on course and that the desired outcomes are attained.

1 INTRODUCTION

The Society of Paediatric Anaesthesiologists of Kenya (SPAK) is dedicated to advancing the practice of paediatric anaesthesia and promoting the welfare of children undergoing surgical procedures. Located at the KMA Centre in Nairobi, SPAK provides a range of educational resources and events aimed at enhancing the knowledge and skills of its members. The organization offers webinars, courses, and a comprehensive library of publications, including the *Journal of SPAK*, designed to keep professionals informed of the latest developments in the field.

SPAK also hosts events, such as the annual congress, enabling members to connect, share insights, and collaborate on best practices. The society places a strong emphasis on community engagement and encourages support through donations. With a commitment to excellence in paediatric anaesthesia, SPAK aims to improve patient care standards and foster a network of dedicated professionals. Joining SPAK offers an opportunity for continued professional development and involvement in a vital area of healthcare.

This *Strategy 2025 – 2030* outlines the society's five-year strategic direction, including its strategic objectives and a framework for monitoring its execution.

1.1 Objectives of SPAK

The Society was established to provide the following services:

- i. Organize regular education and welfare meetings for our members.
- ii. Organize and set standards for training in paediatric anaesthesia.
- iii. Collaboration with other professionals in the provision of paediatric anaesthesia.



1.2 SWOT Analysis

Table 1 summarizes the results of the SWOT (strengths, weaknesses, opportunities, and threats) analysis that was undertaken on the society.

Table 1: SPAK's Strengths, Weaknesses, Opportunities, and Threats.

		ENABLERS	PAINS
Internal		Strengths	Weaknesses
		▪ Unique expertise ▪ Well-defined patient population ▪ Willing partners ▪ Committed members	▪ Few members ▪ Slow growth of paediatric anaesthesiologists ▪ Restricted financial muscle ▪ Non-existent secretariat ▪ Lack of national definition of scope of practice
External		Opportunities	Threats
		▪ Gap in skills among physician and non-physician anaesthetists ▪ Room for growth in numbers in paediatric anaesthesia sub-specialties ▪ No historical baggage ▪ Growing research in paediatric anaesthesia ▪ Growing fellowship trainings in the region	▪ Paediatric anaesthesia is practiced by physician and non-physician anaesthetists ▪ Slow growth of paediatric anaesthesiologists ▪ Skewed distribution of paediatric anaesthesiologists in the country ▪ Lack of recognition of the sub-specialty by potential stakeholders ▪ Being overshadowed by our mother society ▪ Unappreciated value

The Society, having been launched only in June 2023, is still at its infancy stage. Consequently, its membership remains low and is marked with slow growth. These factors impact the practice of the discipline, with physician and non-physician anaesthetists delivering paediatric anaesthesia in most instances, and the leave the Society at a rather weak financial position. However, there are opportunities, as presented in Table 1, which can be leveraged to improve this situation.

2 KEY STRATEGIC ELEMENTS

2.1 Our Mission

“To Educate and Equip Anaesthesia Providers for Excellence in Paediatric Anaesthesia Care.”

2.2 Our Core Values

- **Professionalism & Integrity:** We always provide the highest level of compassionate services. We demonstrate quality, ethical behaviour and accountability in our work and act in the best interest of the people we serve - our patients and their families, our professional colleagues, each other, and our authorities. We treat people with dignity and consideration.
- **Innovativeness:** We constantly strive to redefine the standards of excellence, be adaptive and responsive to change, and improve the delivery of services to our stakeholders.
- **Collaboration:** We provide our services in partnership with patients and their families, other healthcare professionals, and local, regional, and global service providers and professional societies to ensure shared decision-making, coordination, and cooperation.
- **Safety:** We put safety first, hold ourselves accountable for safe outcomes for patients, families, and staff, and ensure the necessary resources to support safety are available.
- **Teamwork:** We harness our internal capabilities, working together and sharing information while attending to our stakeholders.

The mnemonic PICoST¹ will be used for these core values.

2.3 Our Vision

“Excellence in Paediatric Anaesthesia Provision Locally, Regionally, and Globally.”

2.4 Our Motto

“In Safe Hands.”

2.5 Priority Areas of Focus and Desired Outcomes

In order to overcome the challenges and leverage the opportunities identified during the situation analysis and presented in Table 1, SPAK will focus on the priority areas listed in Table

¹ PICoST can be linked to sodium picosulfate/magnesium oxide/citric acid solution, a laxative used to clean out the intestines before surgery or certain procedures (such as colonoscopy, X-rays).

2. Each of these priority areas, which are also referred to as “Pillars of Excellence,” has associated with it a desired outcome that will be its measure of success.

Table 2: Strategic Pillars and Desired Outcomes

Pillar	Desired Outcome
Advocacy	Better MoH & county governments’ support for paediatric anaesthesia.
Member Development	Increased number and capacity building of paediatric anaesthesiologists.
Enhanced Safety of Paediatric Patients	Compliance with WHO/KDHS ² 2022 standards for paediatric care.
Accessibility of Paediatric Anaesthesia Services (incl. Outreach Services)	Paediatric anaesthesia services at all L5 & L6 health facilities in Kenya (incl. paediatric anaesthesia oversighted services).

2.6 The SPAK Strategy House

The foregoing strategic elements are summarized in SPAK Strategy House shown in Figure 1.

² Kenya Demographic and Health Survey

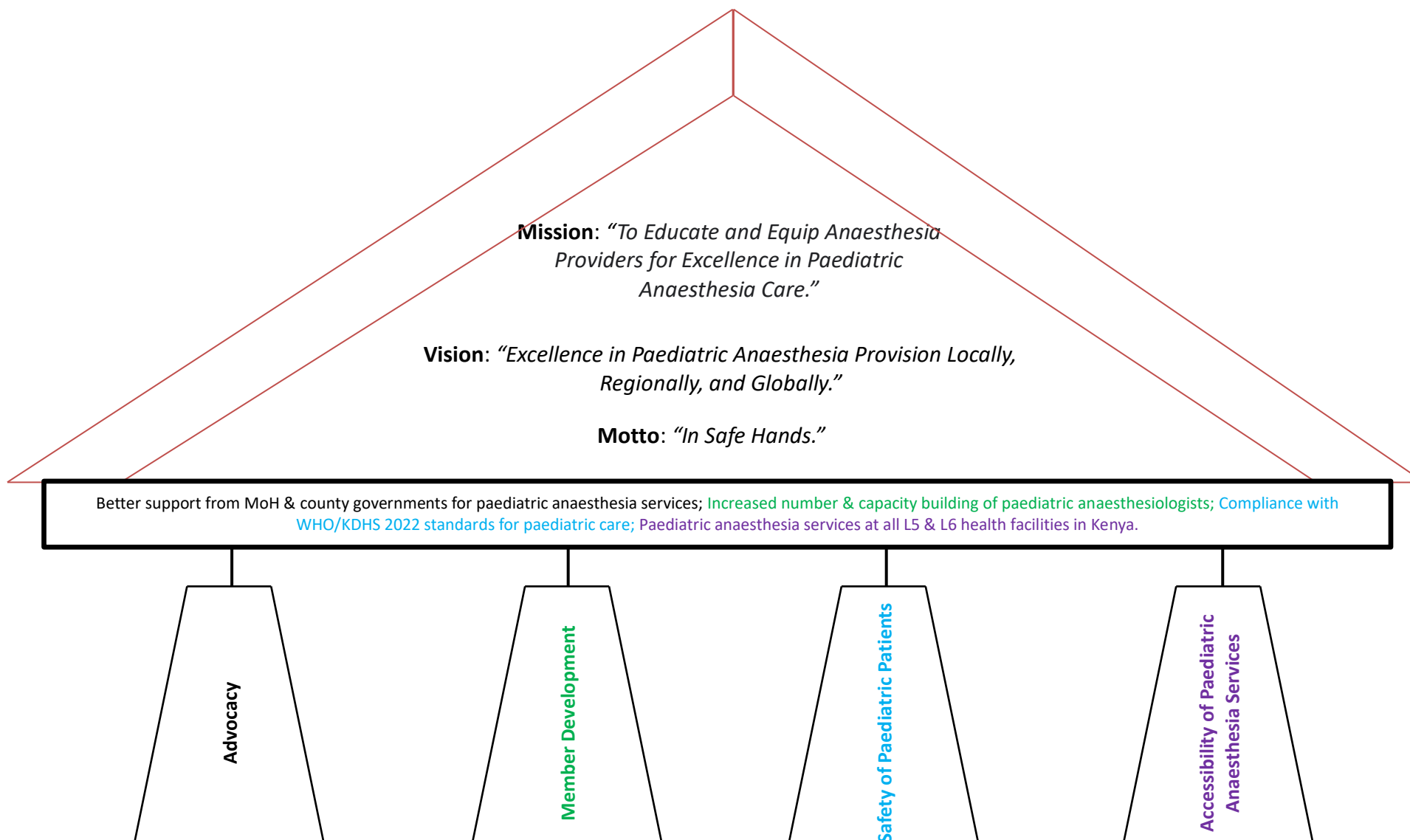


Figure 1: The SPAK Strategy House.

Core Values: Professionalism & Integrity; Innovativeness; Collaboration; Safety; and Teamwork

3 STRATEGIC OBJECTIVES

In order to realize the respective desired outcome for each of the strategic pillars captured in Table 2, the strategic objectives, shown in Table 3, will be implemented over the five-year period.

Table 3: SPAK's Strategic Objectives.

No.	Strategic Objective	Strategic Pillar
SO1	Enhance Strategic Partnerships	Advocacy
SO2	Improve Awareness of Paediatric Anaesthesia	Advocacy
SO3	Enhance Welfare of Members	Member Development
SO4	Lobby National & County Governments for Increased Support for Paediatric Anaesthesia Services (incl. training and posting of paediatric anaesthesiologists, and equipping the facilities offering paediatric services)	Accessibility of Paediatric Anaesthesia Services
SO5	Enhance Training of Paediatric Anaesthesia Practitioners	Safety of Paediatric Patients
SO6	Broaden/Deepen Knowledge of Paediatric Anaesthesiologists	Member Development
SO7	Grow the Number of Paediatric Anaesthesiologists	Member Development
SO8	Increase the Number of Facilities Adequately Equipped and Accredited to Offer Paediatric Anaesthesia	Safety of Paediatric Patients
SO9	Enhance Oversight of Paediatric Anaesthesia Services	Accessibility of Paediatric Anaesthesia Services

These strategic objectives are linked in a cause-and-effect relationship in the strategy map depicted in Figure 2, which shows SPAK's value creation process. The performance metric(s) for each strategic objective is (are) shown in the red text.

Table 4 presents the implementation matrix for this strategy, capturing the strategic objectives and their respective performance metric(s), performance target(s), strategic initiative(s), and the implementation timelines and estimated budgets for the strategic initiatives.

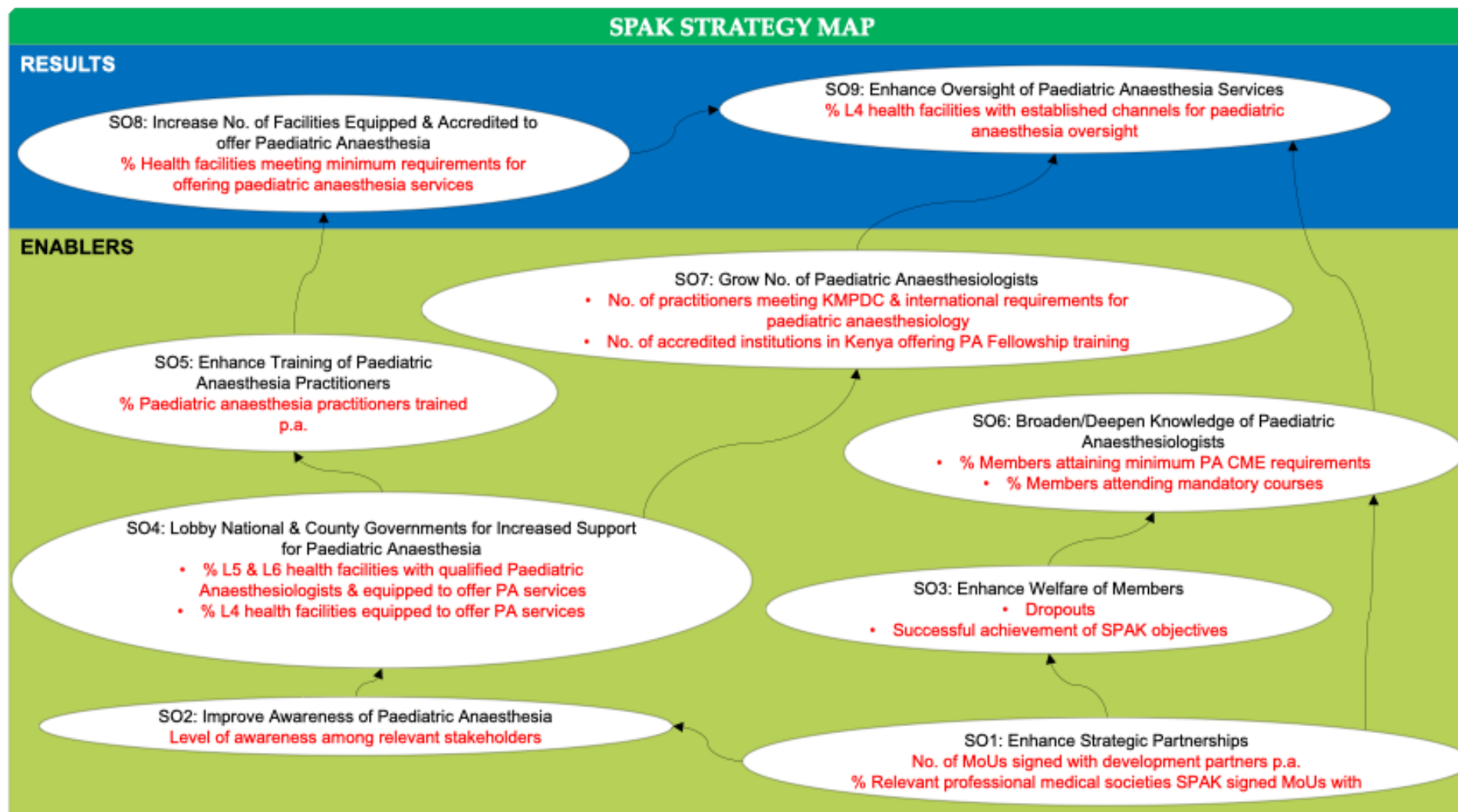


Figure 2: SPAK Strategy Map.

Table 4: Implementation Matrix

Strategic Objective	Performance Metric(s)	Performance Target(s)	Strategic Initiatives	Implementation Timelines for Initiatives	Budgets for Initiatives (Kshs. Millions)
SO1: Enhance Strategic Partnerships	No. of MoUs signed with Development Partners p.a.	Two (2) p.a.	- Identify areas needing support (e.g., training, equipment, & advocacy roles/positions) - Identify suitable development partners - Engage the partners - Renew partnerships as may be appropriate	- April 2025 - June 2025 - July 2025 (& continually afterward) - As they fall due (as recommended by Review Committee)	- Zero - Zero 100K - Zero
	% of Relevant Professional Medical Societies SPAK signed MoUs with	100% (by June 2026)	- Identify professional societies to engage - Enter formal agreements with the professional societies	- June 2025 - June 2026	- Zero 500K
SO2: Improve Awareness of Paediatric Anaesthesia	Level of Awareness Among Relevant Stakeholders (families, medical colleagues, professional societies, administrators, pharmaceutical cos., suppliers, and relevant government departments)	Baseline (X) Target: X+20% p.a.	- Conduct a baseline survey - Facilitate meetings with KMPDC to promote SPAK's agenda - Conduct digital media campaigns highlighting SPAK activities - Engaging MoH & County Govts. to support & facilitate PA services and training	- June 2025 - June 2025 (letter to KMPDC) - June 2025 (quarterly afterward) - September 2025	500K - Zero 100K 100K
SO3: Enhance Welfare of Members	Dropouts	Less than 5% of Membership over plan period (5 yrs.)	Reaching out to other members in their practice regularly	Continually	Zero
	Successful achievement of SPAK objectives/targets	100%	Establish and operationalize a Welfare Team (wellness, webinars, celebrating life occasions, group insurance for	April 2025	1 million p.a.

Strategic Objective	Performance Metric(s)	Performance Target(s)	Strategic Initiatives	Implementation Timelines for Initiatives	Budgets for Initiatives (Kshs. Millions)
			members) – separate from the Executive - Establish and offer psycho-social support to members. - Annual gathering of members - Conducting M&E of activities & performance	- June 2025 - Every June	500K p.a. 400K p.a.
SO4: Lobby National & County Govts for Increased Support for Paediatric Anaesthesia	% of L5 & L6 Health Facilities with Qualified Paediatric Anaesthesiologists	100%	- Conduct a baseline study of PA practice at L5 & L6 health facilities - Engage the CoG - Engage MoH, with KMPDC, to develop policies underpinning the practice of & requisite infrastructure for PA in Kenya - Engage Management of L5 & L6 health facilities to allocate adequate budget for training of Paediatric Anaesthesiologists (a SPAK member with the blessing of the SPAK Secretariat) and equipping the facilities for PA services - Engage Management of L4 health facilities to allocate adequate budget for equipping the facilities for PA services Note: All residents must have undertaken SAFE PAEDS before qualifying	- June 2025 - September 2025 - October 2025	300K 50K 50K
	% of L5 & L6 Health Facilities Equipped to offer PA Services	100%		July 2025	50K
	% of L4 Health Facilities Equipped to offer PA Services	60%		December 2025	

Strategic Objective	Performance Metric(s)	Performance Target(s)	Strategic Initiatives	Implementation Timelines for Initiatives	Budgets for Initiatives (Kshs. Millions)
SO5: Enhance Training of PA ³ Practitioners	% PA Practitioners Trained p.a.	20% p.a.	• Training PA practitioners on SAFE PAEDS (min. 6 p.a.) – 70% p.a. • Train on Resuscitation Course (1 p.a.) • EPALS/APLS (every 2 years) • CME Training (specific to PA): 2 p.a. (1/2-day symposia)	• Every Quarter • Sept. of every year • Biennial (?) • Biannual	• 200K plus sponsorships (need costing for the sponsored courses)
SO6: Broaden/Deepen Knowledge of Paediatric Anaesthesiologists	%Members attaining Minimum PA CME Requirements (4 p.a.) (Note: All CME courses should be CPD-accredited)	100% p.a.	• Advertise available PA CME courses on SPAK’s media channels • Publish the calendar of CPD-accredited CME courses • Audit members’ attendance of CME courses	• Immediately • December (of previous year) • Every November	• 50K p.a.
	% Members Attending Mandatory Courses	100% (within 2 years of the introduction of mandatory course)	• Education Committee to identify mandatory courses by May 2025 and immediately adopt newly notified courses	• May 2025 • Immediately afterwards	
		100% for newly notified mandatory courses			
SO7: Grow No. of Paediatric Anaesthesiologists	No. of Practitioners meeting the KMPDC & International Requirements for Paediatric Anaesthesiology	60	• Active mentorship & enrolment of anaesthesia residents/practicing anaesthesiologists (at least 10) to join PA	• By Year 3 of residency • Continually for practicing anaesthesiologists	• 500K + 500K + 2M

³ PA: Paediatric Anaesthesia

Strategic Objective	Performance Metric(s)	Performance Target(s)	Strategic Initiatives	Implementation Timelines for Initiatives	Budgets for Initiatives (Kshs. Millions)
			- Enhance existing MMed programs to graduate sub-specialists in PA - Work with County Governments (Health CECs) to sponsor training in PA	- By 2027 (discussions with varsities) - Immediately	
	No. of Accredited Institutions in Kenya offering PA Fellowship Training	Three (3)	- Work with MoE, MoH CANECSA to modify (MoH & MoE)/introduce additional modules (CANECSA) training curriculum - Liaise with KMPDC and CUE for accreditation	- December 2026 (MoH & MoE) - July 2025 (CANECSA) - June 2026	200K (total)
SO8: Increase No. of Facilities Equipped & Accredited to offer Paediatric Anaesthesia	% Health Facilities meeting Minimum Requirements for offering PA Services	- L4: 60% - L5: 100% - L6: 100%	- Develop the minimum standards for PA facilities - Develop guidelines & SOPs for out-of-theatre services - Liaise with (lobby) KMPDC for PA Facility accreditation	- April 2025 - June 2025 - July 2025	5 million
SO9: Enhance Oversight of Paediatric Anaesthesia Services	% of L4 Health Facilities with established channels for PA oversight	60%	- Engage KMPDC for the establishment of the oversight channels - All PA practitioners must have undertaken SAFE PAEDS	October 2025	

4 MONITORING & EVALUATION FRAMEWORK

An effective M&E (monitoring and evaluation) framework is crucial for the achievements of the performance targets included in the Implementation Matrix (see Table 4) of the strategic objectives and the desired outcomes of each of the strategic pillars. Therefore, three categories of evaluation, namely quarterly, mid-term, and end-term, will be conducted during the implementation of the strategy.

4.1 Quarterly Evaluations of the Strategy

These will be conducted at the end of every quarter to track the implementation of strategic initiatives in order to:

- a) Determine whether strategic initiatives that should have been implemented during the quarter were actually implemented;
- b) Identify the challenges encountered during the implementation;
- c) Identify effective mitigations for the challenges;
- d) Determine whether any performance targets falling due within the quarter under consideration were achieved;
- e) Identify the lessons learnt during the implementation; and
- f) Reallocate resources as necessary to ensure smooth execution.

The implementation timelines and budgets included in Table 4 will be used as the basis for these evaluations.

4.2 Mid-Term Evaluation of the Strategy

A mid-term evaluation of the Strategy will be conducted in June 2028 to determine:

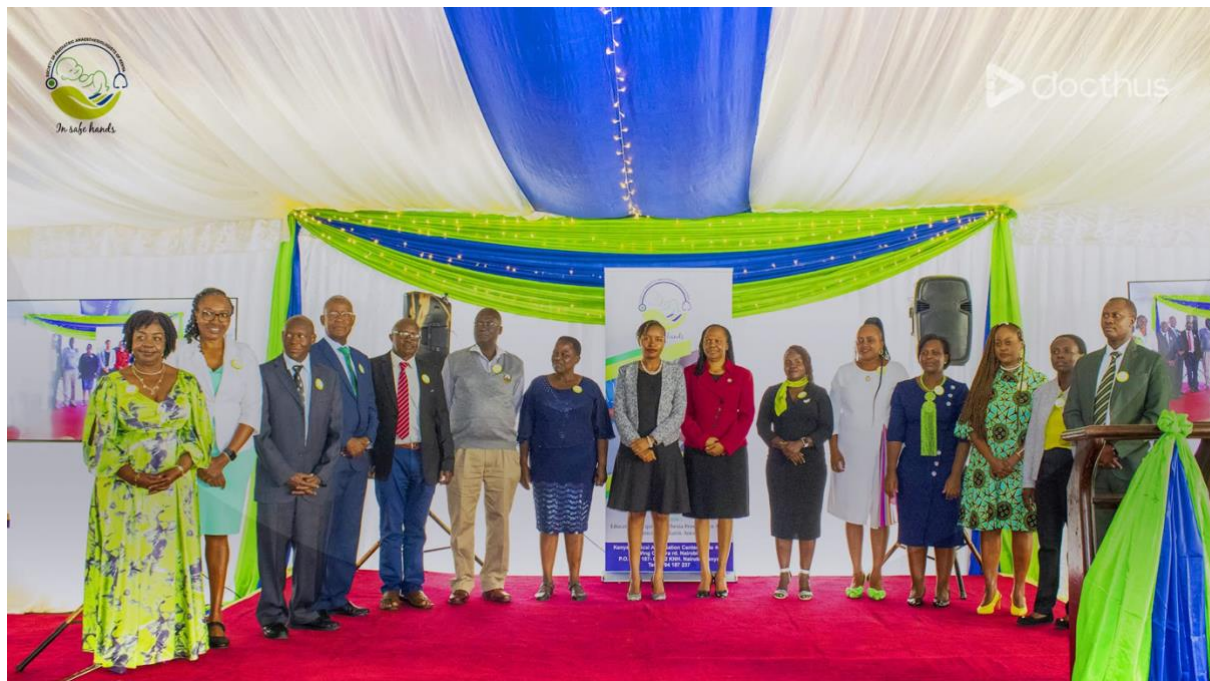
- a) The extent to which the activities (strategic initiatives) undertaken achieved the targets associated with the strategic objectives;
- b) The sustainability of the achievements made;
- c) Overall challenges faced;
- d) Lessons learnt; and
- e) Identify suitable course-correction for the strategy as may be necessary.

The performance metrics and targets included in Table 4 and the respective desired outcome for each strategic pillar will be used as the basis for this evaluation.

4.3 End-Term Evaluation of the Strategy

This will be conducted one (1) month to the end of the Strategy period in the Financial Year 2029/2030. Its main objective will be the overall attainment of performance targets included in Table 4 and the respective desired outcome for each strategic pillar will be used as the basis

for this evaluation. Achievements, challenges, lessons learnt, and recommendations will form the basis for the next cycle of the strategy formulation process.



GLOSSARY

Core Values: Our guiding principles; what we believe in... the ideals, principles, and philosophy at the centre of the organization.

Mission: What we are about; our purpose.

Motto: A short sentence or phrase encapsulating the beliefs or ideals of the organization.

Outcome: Results that are desired by producing outputs; accomplishments.

Performance Metric: An indicator of performance that shows how successful we are in achieving our outcomes.

Performance Target: Desired level of performance for a performance metric.

Strategic Initiative: High-impact projects designed to significantly impact strategic performance.

Strategy Map: The “big picture” of how we create value that demonstrates the key cause-effect relationships among the objectives that make up our strategy.

Strategic Objective: Key strategy component; continuous improvement activity that must be performed.

Strategic Pillar: A priority focus area of the strategy; used to focus member efforts on accomplishing the vision.

Vision: What we want to accomplish in the future; our “picture of the future.”